United Methodist Homes
Skilled Nursing Facility
Policy and Procedure

SUBJECT: ABUSE, NEGLECT, MISAPPROPRIATION OF RESIDENT PROPERTY, EXPLOITATION, AND CRIME

REGULATION:

OBRA: F550; F585; F600; F602-610; F942; F9463; F947

NEW YORK STATE: §415.4, 81.1; NYS Public Health Law (PHL) Section 2803-d

PENNSYLVANIA: §201.3; Older Adults Protective Services Act (OPSA) (1987 Act 79-amended by Act 13 of 1997); Act 70 of 2010

OTHER REFERENCES:

- Centers for Medicare and Medicaid Services (CMS). QSO-22-19-NH- Revised Long-Term Care Surveyor Guidance: Revisions to Surveyor Guidance for Phases 2 & 3, Arbitration Agreement Requirements, Investigating Complaints & Facility Reported Incidents, and the Psychosocial Outcome Severity Guide. June 29, 2022.
- New York State Department of Health (NYSDOH). Nursing Home Incident Reporting Manual. August 2016.
- NYSDOH. NH 22-20 Re: Facility Incident Reporting System. October 18, 2022.
- Pennsylvania Department of Health (PADOH). Mandatory Abuse Reporting. Accessed: June 6, 2023.
- PADOH. Long Term Care Facility Regulations July 1, 2023 Effective Date. February 23, 2023.
- United Methodist Homes Skilled Nursing Facility (UMH) Abuse Prevention-Related Policies.
- UMH Grievance Policy.
- UMH Resident Rights Policy.
- UMH Protecting Resident Privacy and Prohibiting Mental Abuse Related to Photographs and Audio/Video Recordings by Nursing Home Staff Policy.

POLICY STATEMENT:

It is the policy of United Methodist Homes' skilled nursing facilities to honor, support, and enforce the resident's right to be free from abuse, neglect, misappropriation of resident property, and exploitation. This includes but is not limited to freedom from corporal punishment, involuntary seclusion and any physical or chemical restraint not required to treat the resident's symptoms.

FEDERALLY-BASED DEFINITIONS:

"Abuse"- The willful infliction of injury, unreasonable confinement, intimidation, or punishment with resulting physical harm, pain or mental anguish. Abuse also includes the deprivation by an individual, including a caretaker, of goods or services that are necessary to attain or maintain physical, mental, and psychosocial well-being. Instances of abuse of

all residents, irrespective of any mental or physical condition, cause physical harm, pain or mental anguish. It includes verbal abuse, sexual abuse, physical abuse, and mental abuse including abuse facilitated or enabled through the use of technology.

"Agent"- Any person who has been delegated the authority to obligate or act on behalf of a provider.

"Alleged violation"- A situation or occurrence that is observed or reported by staff, resident, relative, visitor, another health care provider, or others but has not yet been investigated and, if verified, could be noncompliance with the Federal requirements related to mistreatment, exploitation, neglect, or abuse, including injuries of unknown source, and misappropriation of resident property.

"Adverse event"- An untoward, undesirable, and usually unanticipated event that causes death or serious injury, or the risk thereof.

"Chemical Restraints"- Any drug that is used for discipline or convenience and not required to treat medical symptoms.

"Convenience"- Any action taken by the facility to control a resident's behavior or manage a resident's behavior with a lesser amount of effort by the facility and not in the resident's best interest.

"Covered individual"- Any individual who is an owner, operator, employee, manager, agent, or contractor of the facility.

"Crime"- An illegal act defined by law of the applicable political subdivision where the facility is located. Examples of situations that would likely be considered crimes in all subdivisions would include but are not limited to:

a. Murder;
b. Manslaughter;
c. Rape;
d. Assault and battery;
e. Sexual abuse;
f. Theft/Robbery;
g. Drug diversion for personal use or gain;
h. Identity theft; and
i. Fraud and forgery.

"Criminal sexual abuse"- In the case of "criminal sexual abuse" which is defined in section 2011(19)(B) of the Act, serious bodily injury/harm shall be considered to have occurred if the conduct causing the injury is conduct described in section 2241 (relating to aggravated sexual abuse) or section 2242 (relating to sexual abuse) of Title 18, United States Code, or any similar offense under State law. In other words, serious bodily injury includes sexual intercourse with a resident by force or incapacitation or through threats of harm to the resident or others or any sexual act involving a child. Serious bodily injury also includes sexual intercourse with a resident who is incapable of declining to participate in the sexual act or lacks the ability to understand the nature of the sexual act.

"Discipline"- Any action taken by the facility for the purpose of punishing or penalizing residents.

"Evidence"- Anything that will help reconstruct the incident, identify people involved and/or identify the responsible persons.

"Exploitation"- Taking advantage of a resident for personal gain through the use of manipulation, intimidation, threats, or coercion.

"Freedom of movement"- Any change in place or position for the body or any part of the body that the person is physically able to control.

"Injury of unknown source"- An injury classification used when all of the following criteria are met:

a. The source of the injury was not observed by any person; and
b. The source of the injury could not be explained by the resident; and
c. The injury is suspicious because of the extent of the injury or the location of the injury (e.g., the injury is located in an area not generally vulnerable to trauma) or the number of injuries observed at one particular point in time or the incidence of injuries over time.

"Involuntary seclusion"- Separation of a resident from other residents or from her/his room or confinement to her/his room (with or without roommates) against the resident's will, or the will of the resident's legal representative.
[Note: This excludes emergency or short term monitored separation from other residents and may be permitted if used for a limited period of time as a therapeutic intervention to reduce agitation until professional staff can develop a plan of care to meet the resident's needs.]

"Law enforcement"- The full range of potential responders to elder abuse, neglect, and exploitation including:

a. Police, sheriffs, detectives, public safety officers;
b. Corrections personnel;
c. Prosecutors;
d. Medical examiners;
e. Investigators; and
f. Coroners.

"Mental abuse"- Abuse occurring through either verbal or nonverbal conduct which causes or has the potential to cause the resident to experience humiliation, intimidation, fear, shame, agitation, or degradation.

"Misappropriation of resident property"- The deliberate misplacement, exploitation, or wrongful, temporary, or permanent use of a resident's belongings or money without the resident's consent.

"Mistreatment"- The inappropriate treatment or exploitation of a resident.

"Neglect"- The failure of the facility, its employees or service providers to provide goods and services to a resident that are necessary to avoid physical harm, pain, mental anguish, or emotional distress. Neglect occurs when the facility is aware of, or should have been aware of, goods or services that a resident(s) requires but the facility fails to provide them to the resident(s), resulting in, or may result in, physical harm, pain, mental anguish, or emotional distress. Examples of individual failures include, but are not limited, to the following:

a. Failure to provide sufficient, qualified, competent staff, to meet resident's needs;
b. Failure to provide orientation and/or training to staff;
c. Failure to provide training on new equipment or new procedures or medications required for the care of a specified resident or required due to changes in acceptable standards of practice;
d. Failure to oversee the implementation of resident care policies;
e. Failure to provide supervision and/or monitoring of the delivery and implementation of care;
f. Failure of staff to implement resident interventions, even when residents are assessed and interventions are identified in the care plan;
g. Failure to identify, assess, and/or contact a physician and/or prescriber for an acute change in condition, and/or a change in condition that requires the plan of care to be revised to meet the resident's needs in a timely manner;
h. Failure to ensure staff respond correctly to medical or psychiatric emergencies;
i. Failure to implement an effective communication system across all shifts for communicating necessary care and information between staff, practitioners, and resident representatives;
j. Failure to monitor and/or provide adequate supervision to assure that environmental hazards are not present including but not limited to:
    i. Access to hot water of sufficient temperature to cause tissue injury;
    ii. Non-functioning call system without a compensatory action;
    iii. Improper handling/disposal of hazardous materials, chemicals and waste;
    iv. Infestation by insects/rodents;
k. Failure to provide adequate monitoring and supervision, if smoking is allowed;
l. Failure to meet financial obligations for the delivery of care and the maintenance of the facility (e.g. payment for staff, utilities, contractors);
m. Failure of the QAPI committee to develop and implement appropriation action plans to correct identified quality deficiencies;
n. Failure of administration to effectively and efficiently use its resources to attain or maintain the highest practicable physical, mental, and psychosocial well-being; and
o. Failure to provide oversight of medical services that are provided in the facility.

"Physical abuse"- Hitting, slapping, pinching, and kicking. It also includes controlling behavior through corporal punishment.

"Physical restraint"- Any manual method, physical or mechanical device, material, or equipment attached or adjacent to the resident's body that the individual cannot remove easily and that restricts freedom of movement or normal access to one's body.

"Political subdivision"- A city, county, township or village, or any local unit of government created by or pursuant to State law.

"Section 1150B of the Social Security Act (the Act)"- Established under the Elder Justice Act (EJA), Section 6703 of the Patient Protection and Affordable Care Act of 2010 (ACA), this section of the Social Security Act outlines specific requirements for the facility and its covered individuals related to crimes committed against a resident of that facility. Key requirements include the responsibility of the facility to:
a. Notify each covered individual initially and annually of their obligation to report any reasonable suspicion of crimes committed against a resident;
b. Ensure there is no retaliation against an individual who lawfully reports a reasonable suspicion of a crime under the Act; and

c. Conspicuously post, in an appropriate location, of a notice for its employees specifying the employees' right to file a complaint with the SA if they feel the facility has retaliated against an employee or individual who reported a suspected crime under the statute, and how to file such a complaint with the SA.

"Serious bodily injury"- An injury involving extreme physical pain, substantial risk of death, protracted loss or impairment of the function of a bodily member, organ, or mental faculty, or requiring medical intervention such as surgery, hospitalization, or physical rehabilitation. Serious bodily injury also is considered to have occurred when an injury results from criminal sexual abuse.

"Sexual abuse"- Non-consensual sexual contact of any type with a resident.

"Verbal abuse"- The use of oral, written or gestured language that willfully includes disparaging and derogatory terms to residents or their families, or within their hearing distance, regardless of their age, ability to comprehend, or disability. Examples include, but are not limited to:

a. threats of harm; and
b. saying things to frighten a resident, such as telling a resident that he/she will never be able to see his/her family again.

"Willful"- The individual must have acted deliberately, not that the individual must have intended to inflict injury or harm.

NEW YORK STATE DEFINITIONS:

"Mental abuse"- Abuse including, but not limited to, abuse that is facilitated or caused by nursing home staff taking or using photographs or recordings in any manner that would demean or humiliate a resident(s).

"Misappropriation"- The theft, unauthorized use or removal, embezzlement or intentional destruction of the resident's personal property including, but not limited to: money, clothing, furniture, appliances, jewelry, works of art and such other possessions and articles belonging to the resident regardless of monetary value.

"Mistreatment"- The inappropriate use of medications, inappropriate isolation or inappropriate use of physical or chemical restraints on a resident of a residential health care while the resident is under the supervision of the facility.

"Neglect"- The failure to provide timely, consistent, safe, adequate and appropriate services, treatment and/or care to a resident while the resident is under the supervision of the facility including, but not limited to:

a. Nutrition;
b. Medication;
c. Therapies;
d. Sanitary clothing and surroundings; and
e. Activities of daily living.

"Physical abuse"- Inappropriate physical contact with a resident of a residential health care facility, while the resident is under the supervision of the facility, which harms or is likely to harm the resident. Inappropriate physical contact includes, but is not limited to, striking, pinching, kicking, shoving, bumping and sexual molestation.

PENNSYLVANIA DEFINITIONS:

"Abuse"- The willful infliction of injury, unreasonable confinement, intimidation, or punishment with resulting physical harm, pain or mental anguish. Abuse also includes the deprivation by an individual, including a caretaker, of goods or services that are necessary to attain or maintain physical, mental, and psychosocial well-being. Instances of abuse of all residents, irrespective of any mental or physical condition, cause physical harm, pain or mental anguish. It includes verbal abuse, sexual abuse, physical abuse, and mental abuse including abuse facilitated or enabled through the use of technology. Willful, as used in this definition of abuse, means the individual must have acted deliberately, not that the individual must have intended to inflict injury or harm.

"Chemical restraint"- Includes psychopharmacologic drugs that are used for discipline or convenience and not required to treat medical symptoms.

"Involuntary seclusion"- Separation of a resident from other residents or from the resident's room or confinement with or without roommates against the resident's will, or the will of the resident's representative, excluding emergency or short term monitored separation from other residents for a limited period of time as a therapeutic intervention to reduce agitation until professional staff can develop a plan of care to meet the resident's needs.

"Mental abuse"- Includes humiliation, harassment, threats of punishment or deprivation.

"Misappropriation of resident property"- The deliberate misplacement, exploitation, or wrongful, temporary, or permanent use of a resident's belongings or money without the resident's consent."

"Mistreatment"- Inappropriate treatment or exploitation of a resident.

"Neglect"- Failure of the facility, its employees, or service providers to provide goods and services to a resident that are necessary to avoid physical harm, pain, mental anguish, or emotional distress.

"Older Adults Protective Services Act (OAPSA) ("PA ACT 13 of 1997")- Includes Commonwealth-specific abuse prevention and reporting guidelines, including screening criteria and mandatory abuse reporting guidelines. OPSA abuse reporting requires employees or the administrator of a facility who has reasonable cause to suspect that a recipient is a victim of any type of abuse to immediately make an oral report to the Area Agency on Aging (AAA). In addition to reporting to AAA, oral reports must be made to the Pennsylvania Department of Aging (PDA) the Ombudsman's office and local law enforcement for suspected abuse involving sexual abuse, serious physical injury, serious bodily injury or if a death is suspicious.

"Physical abuse"- Includes hitting, slapping, pinching, and kicking. The term also includes controlling behavior through corporal punishment.

"Physical restraint"- Any manual method, physical or mechanical device, equipment or material that is attached or adjacent to the resident's body, cannot be removed easily by the resident, and restricts the resident's freedom of movement or normal access to the resident's body.

"Serious Bodily Injury"- An injury which creates a substantial risk of death or which causes serious permanent disfigurement or protracted loss or impairment of the function of a body member or organ.

"Sexual abuse"- Non-consensual contact of any type with a resident, including sexual harassment, sexual coercion or sexual assault.

"Verbal abuse"- Any use of oral, written or gestured language that willfully includes disparaging and derogatory terms to residents or their families, or within their hearing distance, regardless of their age, ability to comprehend or disability. Examples of verbal abuse include:

a. Threats of harm.
b. Saying things to frighten a resident, such as telling a resident that the resident will never be able to see his family again.

PROCEDURES:

SCREENING OF EMPLOYEES

1. Ensure all potential employees and individuals engaged under contract are screened in accordance with all pertinent State and Federal regulations to ensure the facility does not employ or otherwise engage individuals who:

a. Have been found guilty of abuse, neglect, exploitation, misappropriation of property, or mistreatment by a court of law;
b. Have had a finding entered into the State nurse aide registry concerning abuse, neglect, exploitation, mistreatment of residents or misappropriation of their property; or
c. Have a disciplinary action in effect against his or her professional license by a state licensure body as a result of a finding of abuse, neglect, exploitation, mistreatment of residents or misappropriation of resident property.

[Note: Screening shall include, but is not limited to, attempting to obtain employment history information from previous employers and/or current employers, criminal background checks as required by regulation, and reviewing documentation of disciplinary actions from licensing or registration boards and other registries as indicated. Affiliated academic institutions, agencies, or other companies will provide such screening prior to assignment in facility.]
[PA note: Ensure screening and employment criteria is consistent with OPSA criteria.]
RESPONSIBILITY: Human Resources Director or Designee

TRAINING AND NOTIFICATION OF RIGHTS AND RESPONSIBILITIES

1. Provide initial (through orientation), on-going yearly, and as needed training and education for nursing home staff (employees, consultants, contractors, volunteers, and other caregivers who provide care and services to residents on behalf of the facility) related to the prohibition of all forms of abuse, neglect, exploitation, and crime. Such training would include, but is not limited to:

a. Rights guaranteed to residents under Federal and State law, including the right to be free from abuse, neglect, misappropriation of resident property, and exploitation (including, but not limited to verbal, mental, sexual, or physical abuse, corporal punishment, or involuntary seclusion);
b. The facility expectation that all staff protect, promote, and respect such rights;
c. Appropriate interventions to deal with aggressive and/or catastrophic reactions of residents;

d. Dementia management and resident abuse prevention techniques;
e. How to recognize and report signs of burnout, frustration, and stress that may lead to abuse;
f. How to identify, prevent, and respond to indicators, behaviors, and activities that constitute abuse, neglect, exploitation, misappropriation of resident property;
g. Reporting responsibilities and procedures for reporting incidents of abuse, neglect, exploitation, or the misappropriation of resident property, including timeframes for reporting, and to whom staff and others must report their knowledge related to any alleged violation without fear of retaliation;
h. Reporting responsibilities and procedures for reporting reasonable suspicion of a crime against a resident including timeframes for reporting, and to whom staff and others must report their knowledge along with examples of crimes;
i. The reporter's right to be free from liability (civil or criminal), retaliation, and discrimination when a report of abuse, neglect, misappropriation, exploitation, and/or crime is made in good faith;
j. Potential consequences to related to the abuse, neglect, misappropriation of resident property, exploitation, and crime including, but not limited to:
    i. Disciplinary action up to and including termination (for employees);
    ii. Reporting of such violations to the primary employer and potential prohibition of future utilization of the individual's services at the United Methodist Homes (for agency/contract employees);
    iii. Prohibition from volunteer or training at the facility (for volunteers and students); and
    iv. Reporting of such violations/failure(s) to State and Federal agencies in accordance with all pertinent laws and regulations;
k. Consequences related to failure to report abuse, neglect, misappropriation of resident property, and exploitation includes:
    i. Disciplinary action up to and including termination of employment for employees; and
    ii. A finding of professional misconduct and possible fine for any licensed staff by their respective licensing board; and
l. Failure to report a crime results in civil and criminal liability for covered individuals and is punishable by fines, jail time, or both.

[Note: For contract agency staff, the provision of training in accordance with facility policy may be assigned to the contract agency.]
RESPONSIBILITY: Staff Development Coordinator or Designee

2. Ensure postings are present in conspicuous locations in the facility regarding:
a. Resident rights, including their right to be free from abuse, neglect, misappropriation, exploitation, and crime;
b. The procedure for reporting suspected abuse and suspicion of a crime, and, for covered individuals, the right to be free retaliation and/or discrimination related to reports made in good faith; and
c. A posting in an area accessible to residents, employees and visitors is the name, title, location and telephone number of the individual in the nursing home that is responsible for receiving complaints and ensuring a complaint investigation is completed.

RESPONSIBILITY: Administrator or Designee

3. Consider conducting periodic drills across all levels of staff and all shifts to assure that covered individuals understand the reporting requirements.
RESPONSIBILITY: Administration

PREVENTION STRATEGIES

1. In addition to the provision of nursing home staff training, provide residents, resident representatives, and families with information on how and to whom they may report concerns, incidents and grievances without the fear of retribution, and provide feedback regarding the concerns that have been expressed.
RESPONSIBILITY: Administrator, Grievance Official, Social Worker, or Designee

2. Ensure that the resident is free from physical or chemical restraints imposed for purposes of discipline or convenience and that are not required to treat the resident's medical symptoms. When the use of restraints is indicated, use the least restrictive alternative for the least amount of time and document ongoing re-evaluation of the need for restraints.
RESPONSIBILITY: IDT

3. Identify, correct, and intervene in situations in which abuse, neglect and/or misappropriation of resident property is more likely to occur. This includes an analysis of:

a. Features of the physical environment that may make abuse and/or neglect more likely to occur, such as secluded areas of the facility;
b. The deployment of staff on each shift in sufficient numbers to meet the needs of the residents, and assure that the staff assigned have knowledge of the individual residents' care needs;
c. Risk for and indications of staff burnout;
d. The supervision of staff to identify inappropriate behaviors, such as using derogatory language, rough handling, ignoring residents while giving care, directing residents who need toileting assistance to urinate or defecate in their beds; and
e. The assessment, person-centered care planning, and monitoring of residents with needs and behaviors which might lead to conflict or neglect, such as residents with a history of aggressive behaviors, residents who have behaviors such as entering other residents' rooms, residents with self-injurious behaviors, residents with communication disorders, those that require heavy nursing care, and/or are totally dependent on staff.

[Note: Intervention should be immediate when inappropriate staff behavior is identified. Leadership must ensure the safety and well-being of residents by taking the appropriate steps to address the inappropriate behavior prior to allowing the individual to resume contact with residents.]
RESPONSIBILITY: Administrator; Department Managers; Director of Quality Management, Grievance Official; Supervisors

IDENTIFICATION

1. Be aware of indicators of abuse, neglect, misappropriation of resident property, exploitation, and crime.

a. Examples of indicators of possible abuse, neglect, exploitation, mistreatment or crime include:
    i. A statement that abuse, neglect, mistreatment or a crime committed against a resident has occurred;
    ii. Presence of a physical injury or pain inconsistent with the history or course of treatment; and
    iii. Visual or auditory observation of an act of abuse, neglect, mistreatment or crime.
b. Examples of clinical indicators of possible physical abuse include:
    i. Cuts, lacerations, puncture wounds;

ii. Sprains, dislocations, fractures, or broken bones without medical explanation;
iii. Bruises, welts, discoloration;
iv. Bruises or abrasions on bilateral areas of the body or multiple in various stages of healing;
v. Bruises or abrasions in the shape of a hand or object;
vi. Injury that is incompatible with history;
vii. Pain that is incompatible with history;
viii. Absence of hair and/or hemorrhaging below scalp;
ix. Burns (may be caused by cigarettes, caustics, acids, friction, or contact with other objects);
x. Fear of caregiver(s);
xi. Sudden change in behavior; and
xii. A resident report of physical abuse.

c. Examples of clinical indictors of possible sexual abuse include:
i. Bruises around breasts or genital area;
ii. Bruising on inner thighs;
iii. Unexplained venereal disease, genital infection, or pregnancy;
iv. Unexplained genital, vaginal or anal bleeding, pain or irritation;
v. Torn, stained, or bloody underclothing;
vi. Fear of being alone with caregiver(s);
vii. Difficulty in walking or sitting without evidence of musculoskeletal disease;
viii. Presence of semen on or near a resident; and
ix. Resident report of sexual abuse.

d. Examples of clinical indicators of neglect include:
i. Dehydration and/or malnourishment/weight loss without illness-related cause;
ii. Untreated bed sores;
iii. Unattended or untreated health problems;
iv. Poor personal hygiene; and
v. Resident report of being neglected.

e. Examples of indicators of financial or material exploitation include:
i. Sudden changes in bank account or banking practice, including an unexplained withdrawal of large sums of money by a person accompanying the elder;
ii. The inclusion of additional names on an elder's bank signature card;
iii. Abrupt changes in a will or other financial documents;
iv. Unexplained disappearance of funds or valuable possessions;
v. Discovery of a resident's signature being forged for financial transactions or for the titles of his/her possessions;
vi. Sudden appearance of previously uninvolved relatives claiming their rights to an elder's affairs and possessions;
vii. Unexplained sudden transfer of assets to a family member or someone outside the family; and
viii. Residents report of financial exploitation.

f. Injuries that meet the definition for injuries of an unknown source (origin)

[Note: The list of indicators is NOT all inclusive.]
RESPONSIBILITY: All Staff

2. Report all witnessed acts, suspicions, or observed indicators that may constitute abuse, neglect, misappropriation of resident property, exploitation, and crime to the supervisor

immediately and follow additional procedures specified in the "Investigation" and "Reporting" sections.
[Note: If the act or indicator involves the supervisor, report the concern to the department director or administrator.]
RESPONSIBILITY: All Staff

3. Thoroughly investigate indicators and allegations of abuse, neglect, misappropriation of resident property, exploitation, and crime. Identify probable causes and take appropriate corrective actions.
RESPONSIBILITY: Administrator; Director of Nursing Services; Nursing Supervisor; Grievance Official; Designee

INVESTIGATION

1. Refer to "Appendix A: Investigation Key Elements and Process Guide" to aide in the investigation process.
RESPONSIBILITY: Administrator; Participating Staff

2. Upon receipt of an allegation of abuse, neglect, or mistreatment, or related crime, take the following steps immediately:

a. Proceed to the incident scene as promptly as possible.
b. Protect/ensure the safety of the victim/resident.
c. In the event of resident injury:
    i. Assess the injury and arrange for immediate medical care if needed;
    ii. Document the type of injury (nature, location and size), possible cause(s), position of furniture, equipment and/or environmental issues that may be a factor in the incident;
    iii. Take photos if indicated;
    iv. Arrange for the resident to be examined by a physician if indicated;
d. Complete and document a physical assessment of the resident in the medical record.
e. Inform the staff to refrain from questioning the resident and/or witnesses.
f. Protect the scene- preserve physical evidence as able.
g. Observe the scene, take notes with date and time.
h. Locate and speak with the people involved in any way with the incident (i.e. person who found the resident or to who the incident was first reported, people near the incident scene, staff responsible for care of the resident, nurse responsible for the unit).
    i. Interview these individuals while their recollections are fresh.
    ii. Make clear notes.
i. Ensure the nursing director and administrator are notified.
j. If the alleged perpetrator(s) is known and is on-site, remove the individual from work pending the facility's investigation.
k. Notify law enforcement immediately if indicated (e.g., clear evidence that a serious crime has occurred; significant threat to the safety of the involved resident(s) and/or other building occupants remains; etc.).

RESPONSIBILITY: Charge Nurse; Nursing Supervisor

3. For all allegations, begin a formal investigation immediately. Observe the scene, collect evidence, take notes with date and time.
RESPONSIBILITY: Administrator; Nursing Management; Designee

4. Determine actual/potential witnesses, conduct interviews (resident(s), staff and third parties) in accord with the above protocols.
RESPONSIBILITY: Administrator; Nursing Management; Designee

5. Obtain written statements, signed and dated by the writer (include shift and title if an employee), from all involved parties (resident(s), witness(s), accused).
RESPONSIBILITY: Administrator; Nursing Management; Designee

6. Review the medical record, plan of care and all relevant documents.
RESPONSIBILITY: Administrator; Nursing Management; Designee

7. Using CMS guidelines, determine the presence of intent by the aggressor.
RESPONSIBILITY: Administrator; Nursing Management; Designee

8. Obtain photographic evidence, if appropriate.
RESPONSIBILITY: Administrator; Nursing Management; Designee

9. If there is reason to suspect that an employee(s) committed an act of abuse, neglect, mistreatment, crime or other serious misconduct, ensure the individual is suspended from work until the investigation is complete.
RESPONSIBILITY: Administrator; Human Resources; Nursing Management; Designee

10. If there is reason to suspect a visitor committed an act of abuse, neglect, mistreatment, crime or other serious misconduct, ensure the individual is suspended from facility entry until the investigation is complete.
RESPONSIBILITY: Administrator; Nursing Management; Designee

11. Notify the resident/resident's responsible party of the incident and that an investigation is being conducted.
RESPONSIBILITY: Nursing Management; Designee

12. Ensure the executive director, chief operating officer, and vice-president of clinical services are made aware of incidents as indicated (e.g., incidents determined to meet the criteria for abuse, neglect, misappropriation, mistreatment, and/or crime; incidents under investigation that are anticipated to meet the criteria for abuse, neglect, misappropriation, mistreatment, and/or crime; etc.).
RESPONSIBILITY: Executive Director; Administrator; Designee

13. Ensure staff members are aware not to give statements to the media should related media activity occur.
    a. Corporate leadership will designate a spokesperson through whom all medical inquiries will flow and information will be released.

RESPONSIBILITY: Executive Director; Administrator; Nursing Management; Designee

14. In collaboration with the administrator, review findings on an ongoing basis and complete a report, outlining all facts, actions taken, and conclusions.
RESPONSIBILITY: Administrator; Director of Nursing; Designee

15. Notify the resident/resident's responsible party of the results of the investigation as able, maintaining confidentiality of information as required by regulation and law.
RESPONSIBILITY: Administrator; Director of Nursing; Designee

16. Over the course of the investigation, ensure timely reporting to the department of health and/or law enforcement occurs in compliance with regulatory and legal requirements (refer to the "Reporting" section).
RESPONSIBILITY: Administrator; Director of Nursing; Designee

17. Take disciplinary and corrective actions as indicated. If the investigation concludes that employee(s) committed abuse, neglect, mistreatment or crime against a resident, terminate the employee(s).
RESPONSIBILITY: Administrator; Department Director; Human Resources

REPORTING

Initial Internal Reporting-

1. Immediately report all possible incidents of abuse, neglect, misappropriation, exploitation, mistreatment, and crime to the Charge Nurse or Nursing Supervisor upon discovery of the incident as outlined in the "Investigation" section.
[Note: Reports of abuse, neglect, misappropriation, exploitation, mistreatment, and crime are handled in a manner intended to protect the confidentiality of the reporter. Failure to report is not justified by claiming "resident confidentiality."]
RESPONSIBILITY: All Staff

2. Notify the Director of Nursing or the designated on-call management/administrative staff and the Administrator. Take immediate investigation steps as specified in the abuse investigation policy and procedure.
RESPONSIBILITY: Charge Nurse or Nursing Supervisor

External Reporting-

1. Refer to and follow the guidance in "Appendix B: CMS SOM Quick Reference Abuse Reporting Guides- Identification and Reporting of Alleged Violations" and "Appendix C: CMS SOM Quick Reference Abuse Reporting Guides- General Reporting Requirement Chart", amended by UMH to include additional State and Commonwealth reporting information, for guidance in determining what incidents are reportable, who to report to, and timelines for reporting.
RESPONSIBILITY: Administrator; All Covered Individuals

2. Ensure all required reports are completed and submitted in accordance with regulatory requirements and DOH/agency protocols. Key contact information for reporting includes:

- *New York Reporting Contacts*-
    - Department of Health online incident reporting: Nursing Home Facility Incident Report.
    - Department of Health incident backup reporting line: 1-888-201-4563 (for use in case of an emergency such as loss of internet or computer service.)
    - Local law enforcement: the Broome County Sherriff's office: 1-607-778-1911
- *Pennsylvania Reporting Contacts*-
    - Department of Health online incident reporting: Event Reporting System.
    - Pennsylvania Department of Health (local field office): 1-570-963-4331
    - Area Agency on Aging: 1-570-822-1158
    - Ombudsman office: 1-570-550-3811
    - Local law enforcement: the Jenkins Township Police Department: 1-570-654-1281

RESPONSIBILITY: Administrator; Director of Nursing; Designee

3. If a licensed employee is determined to have committed abuse, neglect, mistreatment, misappropriation, or a crime against a resident, notify the disciplinary board for the appropriate licensing agency (e.g., nursing; medical; therapy).
RESPONSIBILITY: Administrator; Director of Nursing; Designee

POLICY AND PROCDURE: ABUSE, NEGLECT, MISAPPROPRIATION OF RESIDENT PROPERTY, AND EXPLOITATION

DATE: 6-2023 (REV)

APPROVED BY:

Ronald Patti — 06/14/2023
Chief Operating Officer — Date

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S Valdo — 06/23/2023
Medical Director — Date

REVIEWED/REVISED:

November 2011; January 2012; February 2012; November, 2016; June 12, 2023

Appendix A:
Investigation Key Elements and Process Guide

Overall Investigation Process and Special Considerations

Process Objectives:

- To determine if abuse, mistreatment, neglect, misappropriation or a crime occurred;
- To assure compliance with statutes and regulations;
- To improve quality of care;
- To identify areas for system improvement;
- To identify and correct vulnerabilities;
- To assist with management of risk; and
- To determine if the concern was a result of an individual(s) action or a facility practice.

Overview:

- Protect the resident(s), remove them from harm;
- Protect the scene- preserve any physical evidence;
- Collect evidence (ask who, what, when, where, and how);
- Determine presence of intent by aggressor;
- Determine your witnesses;
- Conduct interviews;
- Documentation (incident reporting form, medical records, witness/interview statements);
- Determine if abuse, neglect, mistreatment or crime has occurred;
- Document your conclusion;
- Protection of residents from harm during an investigation;
- Prohibit alleged perpetrators from any contact with residents;
- Arrange for supportive counseling (social services, chaplaincy, other professional services) as needed;
- Instruct all staff members caring for the resident to be supportive of the resident, without revealing confidential details of the investigation.
- Resident assessment and monitoring:
    - Ensure adequate post-event assessment of the resident who has incurred actual or potential physical harm;
    - In all instances of alleged abuse, neglect or mistreatment (with the exception of misappropriation of resident property), conduct a thorough physical assessment at the time of discovery and document findings;
    - Monitor the resident every shift for 72 hours post-occurrence and document findings in the resident's medical record; and
    - Continue or discontinue monitoring based on the resident's condition and severity of injury, if applicable.

Special considerations for allegations of sexual abuse:

- Ensure the safety of the resident.
- Be calm and supportive of the resident.
- Until the resident/responsible party determines the course of action to be taken, *do not change the resident's clothing, do not shower or bathe the resident, do not brush*

the resident's teeth, and do not remove any bedding or potential evidence from the scene.

- If the resident is cognitively aware and alert, give him/her the options of the facility notifying family and going to the emergency room for evaluation.
- If the resident is cognitively impaired, notify the health care proxy or legal representative and afford him/her the same options as above.
- If the resident chooses not to go to the emergency room for evaluation, arrange for a physician to examine the resident at the facility.
- Offer counseling to the resident.
- If appropriate, seek assistance from:
    - NY - The Crime Victims Assistance Center
    - PA- Victims Resource Center 1-866-206-9050
- Be supportive of the resident's decisions.
- Make notes of the resident location, of any blood or other stains on clothing, bed linens, furniture, or floor.
- Note location of any bruises, scratches, cuts, or wounds.
- Document the resident's emotional state.
- Assign a facility employee known to the resident to accompany the resident to the hospital.

Special considerations for resident-to-resident sexual abuse:

- Arrange a meeting with appropriate members of the interdisciplinary care team immediately. Include involved resident and family as appropriate.
- Determine whether the act was consensual or non-consensual.
- If consensual, develop appropriate interventions to ensure resident safety and resident rights.
- If non-consensual, proceed with abuse investigation.
- If non-consensual, ensure the aggressor is adequately monitored to protect others and prevent recurrence.

Conducting the investigation:

- The investigation should rule out or confirm abuse, mistreatment, neglect, or crime through a review of supporting evidence, including interviews and statements that offer valid information, observations and record review. **The outcome is determined by facts, based on the evidence and not an opinion which is subjective.**
- Employees may choose to have representation during the interview. However, do
- not allow obtaining representation to unnecessarily delay the interview.
- Employees have the right to decline speaking with authorities and/or furnishing written statements. This organization's expectation of all employees is that they will cooperate with investigations and provide written statements in all cases of possible abuse, neglect, mistreatment or crime.

Interviewing Guide

Interviews defined:

- Obtain statements from parties directly involved and others possessing knowledge of relevant information. This may include staff, residents and third parties.

- A non-accusatory process in which the interviewer asks questions designed to develop factual information.
- Documentation must be signed, dated, indicate shift and should include the individuals title.
- Who, what, where, when, why and how.
- Maintain a list of the questions posed to the individuals being interviewed.
- Obtain statement from the resident(s), if they are able to provide a statement about the incident, whenever possible.

Interview Tips:

- Provide privacy, minimize distractions; listen carefully and be patient.
- Take notes during the interview.
- Enlist a second member of management / administration to assist with interviews and serve as a witness to the interview.
- Introduce yourself(s).
- Try to put the witness at ease.
- Explain the purpose of the interview.
- DO NOT promise to keep information confidential.
- Remind the witness to refrain from discussing the matter outside the interview.
- Do not allow staff members to discuss the incident with the resident until you get there or unless instructed to do so.
- What does staff say about the incident.
- Questions should be open ended, designed to elicit descriptions of observations, interventions, or re-enactments.
- Avoid leading questions.
- Conduct a second interview if needed.

Interviewing Staff:

- Describe interaction with resident.
- Physical demonstration of care delivered.
- Did you see any one physically harm the resident.
- Describe force used, stance of accused and any injury.
- Did you check the chair alarm – was it working.
- Did you see "Patty" use the gait belt.
- What can the resident do independently.
- What did the plan of care require or direct.
- Determine the staff members credibility and reliability.
- Is there any known dissension between the witness and the accused.
- Be certain that the staff member understands questions and that you understand his/her answers.
- Restate the witness's statements to clarify.
- If English language is a barrier, consider obtaining an interpreter.

Interviewing Residents:

- Ask the resident to tell you what happened; state exact words, circumstances and responses.
- Be alert that the resident may be too frightened to offer testimony.

- Re-approach the resident if they are unable to be interviewed for any reason (within 24 hours).
- Determine the resident's credibility and reliability.
- How does the resident(s) feel about the care provided by the accused.
- Be certain that the resident understands questions and that you understand his/her answers.
- Restate the witness's statements to clarify.
- If English language is a barrier, consider obtaining an interpreter.

Interviewing Third Party Witnesses:

- What is the relationship of the witness to the resident/victim.
- How often does the witness visit the facility.
- Was the witness previously employed by the facility.
- Determine the witness's credibility and reliability.
- Is there a history of allegations.
- Be certain that the witness understands questions and that you understand his/her answers.
- Restate the witness's statements to clarify.
- If English language is a barrier, consider using an translation application or obtaining the services of an interpreter.

Reluctant Witness:

- Repeat your objective of obtaining all of the facts of the incident.
- Remind the witness of staff and facility responsibility to protect the residents at all times.
- Stress that management is concerned about the rights of employees not to be falsely or unjustly accused of misconduct, and that only a full and fair investigation can protect this right.
- Do not promise to maintain confidentiality.
- Probe aggressively if necessary to break down obvious inconsistencies or false statements, pointing out these gaps to the witness.

Written Statements:

- Written statements should be legible, signed, dated and indicate shift and job title. The preference is to have individual(s) write their own statement, but if they are unable, transcribe the statement and obtain the individuals signature or have it witnessed.
    - Obtain written statements from the victim, the accused, and from witnesses.
    - Additional staff/witness statements may be necessary with injuries of unknown origin.
    - Additional staff/witness statements may be necessary to determine what care was delivered - was the care provided in accordance with the individualized plan of care.
    - Written statements will need to be obtained in addition to the interview.

<u>Observation Guide</u>

While observing the surroundings or scene where the incident is alleged to have occurred, protect the scene.

- With allegations of abuse, mistreatment, neglect or crime: try to leave the area untouched, as much as possible.
- Preserve any evidence/photographs (photos must be dated, timed and able to correctly identify the subject).
- Make note of resident condition, environmental conditions, and circumstances just prior to the event or discovery

Observation:

- What did you see.
- What did others see.
- What does the environment tell you.
- Is there evidence that the plan of care was followed, or that it was not followed.
- What do you see when you look at or speak to other residents.
- What do you hear when walking through the unit.
- Do you see evidence of potential abuse or concern.

Record Review Guide and Evidence Gathering

Resident Records:

- What was required by the plan of care.
- Was it an appropriate individualized plan of care.
- What are the resident's risk factors.
- Are the risk factors identified in the plan of care.
- Was the plan of care followed.
- What was documented upon discovery of the event.
- What do nurses' reports indicate.
- Were medical practitioner orders followed.
- Was the CNA documentation completed accurately.
- Were facility policies and procedures followed.
- Does the plan of care need to be updated to prevent recurrence.
- Are there indications of undiagnosed medical issues.
- What is the resident's history, is there a history of false allegations.
- What is the residents behavior.
- Does the plan of care reflect actual care needed.
- Does CNA plan of care match the comprehensive care plan.
- Does the CNA plan of care give specific information to the staff.

Staff Records:

- Is there a staff member involved.
- How long has the staff member been employed with the facility.
- What is the staff members performance history.
- Has the staff member been counseled for care issues or failure to follow the plan of care in the past.
- Was care appropriately documented.
- Does the documentation indicate that the plan of care was followed.
- Has the staff member been educated or trained in the area being investigated, and noted in the personnel records.

Case Evidence:

- All evidence used to determine the outcome of the investigation should be relevant to the case.
- If the case is related to a medication, include a hard copy of the proper Medication Administration Record and other related documents in the case file.
- If the case is related to misappropriation is there a description of what is missing, a list of persons with access, a date and time the item(s) was last seen, the value
- of the item.
- If the case is related to financial misappropriation, include related receipts, billing statements, copies of cancelled checks when indicated and available.
- If the case is related to a plan of care violation, include plan of care documents in the case file.
- If extenuating circumstances were identified, (e.g., faulty equipment; insufficient staffing; dissension; disaster conditions; etc.), include supporting evidence in the file.
- Ensure all materials relevant to the outcome are included in the file.

Determination of Willful Action and Intent

- Did the accused deliberately perform an act?
- Ensure key regulatory definitions and guidelines are followed when making determinations (e.g., Federal/State/Commonwealth definitions; SOM interpretive guidance; etc.)

Completing a Thorough Investigation and Determining an Objectively-Supported Conclusion

A thorough investigation:

- Comes to a reasonable conclusion as to how and/or why something occurred.
- Rules out or confirms abuse, neglect, mistreatment or crime by review of supporting evidence, including interviews and statements that offer valid information, observations and record review.
- Outcome is determined by fact, based on the evidence and not on opinion.

At the end of the investigation:

- Upon review of all of the evidence, a conclusion is made. At a minimum, determinations at the end of the investigation answer the following:
    - Did abuse, neglect, mistreatment, misappropriation, or a crime occur?
    - Is there a system, policy, or practice issue that caused or contributed to risk?
    - Is this an isolated incident or a systemic problem?
    - Does the evidence support the outcome?
    - Based upon regulation and law, was the incident reportable?

Final steps

- Ensure all corrective actions have been taken to ensure the safety and well-being of facility occupants (residents; staff; visitors).
- Identify trends.
- Implement and communicate QAPI plans for long term prevention of recurrence.
- Ensure all required reports related to the incident have been made to appropriate agency(s) (refer to policy reporting guidance).

Appendix B:
CMS SOM Quick Reference Abuse Reporting Guides
Identification and Reporting of Alleged Violations

I. Staff to Resident Abuse

All allegations/occurrences of all types of staff-to-resident abuse must be reported to the administrator and to other officials, including the State Survey Agency and adult protective services, where state law provides for jurisdiction in nursing homes. This includes, but is not limited to:

a. All allegations/occurrences of physical, sexual, mental, and verbal abuse, including deprivation of goods and services by staff, and involuntary seclusion perpetrated by staff;
b. Staff taking or distributing demeaning or humiliating photographs or recordings of residents through social media or multimedia messaging; and
c. All reports from residents of abuse perpetrated by staff; allegations **must not** be dismissed on the basis of a resident's cognitive impairment(s).

[Key note: It should not be assumed that every accident or disagreement that occurs between an employee and a resident should be considered to be abuse. Accidents that may not be considered to be abuse include instances such as a staff member tripping and falling onto a resident; or a staff member quickly turning around or backing into a resident that they did not know was there.]

II. Resident to Resident Altercations

Resident-to-resident altercations that must be reported in accordance with the regulations include **any willful action that results in physical injury, mental anguish, or pain**. The tables below includes examples of resident to resident altercations and whether they are required to be reported.

[Key note: This is not an exhaustive list of all reportable types of resident to resident altercations. There may be other incidents that are also reportable.]

Examples of Mental/Verbal Conflict

Required to Report	**Not Required to Report (Unless it rises to the level of what's described in the first column)**
• Intimidation • Bullying- Aggressive behavior in which someone intentionally* and repeatedly causes another resident mental anguish or discomfort** (adapted from the American Psychological Association [2] • Communication that is motivated by an actual or perceived characteristic, such as race, color, religion, sex, disability, or sexual orientation that results in mental anguish or social withdrawal** • Threats of violence • Inappropriate sexual comments that are used in a deliberately* threatening manner • Inappropriate sexual comments that offend, humiliate, or demean a resident**; • Taking and/or distributing demeaning or humiliating photographs or recordings of residents through social media or multimedia messaging	• Non-targeted outbursts • Residents with certain conditions (e.g., Huntington's/Tourette's) who exhibit verbalizations • Arguments or disagreements, which do not include any behavior or communication identified in the "Required to Report" column

* Having a mental disorder or cognitive impairment does not automatically preclude a resident from engaging in deliberate or non-accidental actions.

****** There may be some situations in which the psychosocial outcome to the resident may be difficult to determine or incongruent with what would be expected. In these situations, it is appropriate to consider how a reasonable person in the resident's circumstances would be impacted by the incident.

Examples of Sexual Contact

Required to Report	**Not Required to Report (Unless it rises to the level of what's described in the first**
• Unwanted touching of the breasts or perineal area • A resident who fondles or touches a person's sexual organs and the resident being touched indicates the touching is unwanted through verbal or non-verbal cues • Sexual activities where one resident indicates that the activity is unwanted through verbal or non-verbal cues • Sexual activity or fondling where one of the resident's capacity to consent to sexual activity is unknown • Sexual assault or battery (ex. rape, sodomy, coerced nudity) • Instances where the alleged victim is transferred to a hospital for examination and/or treatment of injuries resulting from possible sexual abuse • Forced observation of masturbation, or pornography • Forced, coerced or extorted sexual activity • Other unwanted actions for the purpose of sexual arousal or sexual gratification resulting in degradation or humiliation of another resident	• Consensual sexual contact between residents who have the capacity to consent to sexual activity • Affectionate contact such as hand holding or hugging or kissing a resident who indicates that he/she consents to the action through verbal or non-verbal cues • Sexual activity between residents in a relationship, married couples or partners, unless one of the residents indicates that the activity is unwanted through verbal or non-verbal cues.

Examples of Physical Altercations That Would Likely NOT Need to Be Reported

- A resident lightly taps another resident to stop irritating behavior or get attention, with no resulting physical injury, mental anguish, or pain.
- A resident who is slow, impedes the pathway of another resident, such as in the dining room, the other resident nudges the resident out of the way to get to his/her table faster, but there is no harm to the victim.
- A resident who swats at another resident who is trying to take some food off his/her plate, and no physical injury, mental anguish, or pain has occurred.

Key notes:
*These examples are not reportable as long as it is not a willful action that results in physical injury, mental anguish, or pain. Every case is fact specific and all facts, circumstances and conditions involving the event/occurrence would need to be examined.
*Even if a physical altercation is not required to be reported, the facility should take into consideration that physical altercations can increase the risk for abuse to occur to residents involved in the altercation, and possibly other residents in the facility. Ensure each incident is thoroughly investigated, the resident appropriately assessed, and the resident's plan of care revised as indicated, and facility systems reviewed as indicated to prevent harm as a result of resident to resident altercations.

Examples of Physical Altercations

Resident-to-resident physical altercations that must be reported include, any willful action that results in physical injury, mental anguish, or pain. Examples include, but are not limited to, the following:

WILLFUL ACTION*

Willful actions include, but are not limited to, the following:

- Hitting
- Slapping
- Punching
- Choking
- Pinching
- Biting
- Kicking
- Throwing objects
- Grabbing
- Shoving

***The action itself was deliberate or non-accidental, not that the individual intended to inflict injury or harm**

That results in

PHYSICAL INJURY

A physical injury resulting from the willful action including, but not limited to, the following:

- Death
- Injury requiring medical attention beyond first aid (such as a cut requiring suturing or an injury requiring transfer to a hospital for examination and/or treatment)
- Fracture(s), subdural hematoma, concussion
- Bruises
- Facial injury(ies), such as broken or missing teeth, facial fractures, black eye(s), bruising, bleeding or swelling of the mouth or cheeks

MENTAL ANGUISH*

Psychosocial outcomes resulting from the willful action including, but not limited to, the following:

- Fear of a person or place or of being left alone or of being in the dark, disturbed sleep, nightmares
- Changes in behavior, including aggressive or disruptive behavior toward a specific person
- Running away, withdrawal, isolating self, feelings of guilt and shame, depression, crying, talk of suicide or attempts

***** There may be some situations in which the psychosocial outcome to the resident may be difficult to determine or incongruent with what would be expected. In these situations, it is appropriate to consider how a reasonable person in the resident's circumstances would be impacted by the incident.

PAIN

Pain resulting from the willful action including, but not limited to, the following:

- Complaints of pain related to the altercation
- Onset of pain evidenced by nonverbal indicators, such as
  - Groaning, crying, screaming
  - Grimacing, clenching of the jaw
  - Resistance to being touched
  - Rubbing/guarding body part

* Having a mental disorder or cognitive impairment does not automatically preclude a resident from engaging in deliberate or non-accidental actions.

III. Reporting Suspicious Injuries of Unknown Source

Examples of Injuries of Unknown Source

Required to Report	**Not Required to Report (Unless it rises to the level of what's described in the first**
• Unobserved/Unexplained fractures, sprains or dislocations • Unobserved/Unexplained injuries that could have resulted from a burn, including blisters or scalds • Unobserved/Unexplained bite marks • Unobserved/Unexplained scratches and bruises found in suspicious locations such as the head, neck, upper chest or back • Unobserved/Unexplained swelling that is not linked to a medical condition • Unobserved/Unexplained lacerations with or without bleeding • Unobserved/Unexplained skin tears in sites found in suspicious locations (e.g., in sites other than the arms or legs) • Unobserved/Unexplained skin tears in patterns (e.g., bilateral, symmetrical skin tears on both arms) • Unobserved/Unexplained patterned bruises that suggest hand marks or finger marks, or bruising pattern caused by an object • Unobserved/Unexplained bilateral bruising to arms, bilateral bruising of the inner thighs, "wrap around" bruises that encircle the legs, arms or torso, and multicolored bruises which would indicate that several injuries were acquired over time. • Unobserved/Unexplained facial injuries, including facial fractures, black eye(s), bruising, or bleeding or swelling of the mouth or cheeks with or without broken or missing teeth • Unobserved/Unexplained bruising or other injuries in the genital area, inner thighs, or breast**s** • Unobserved/unexplained injury requiring transfer to a hospital for examination and/or treatment **NOTE: Any injury that is explained and appears to be a result of abuse must be reported.**	• Bruising in an area where the resident has had recent medical tests/lab draws and there is no indication of abuse or neglect • Injuries where the resident was able to explain or describe how he/she received the injury as long as there is no other indication of abuse or neglect • Injuries that were witnessed by staff, where there is no indication of abuse or neglect **NOTE: Even if the injury is not one that requires a report, the facility should adequately assess and monitor the resident, notify the physician/resident representative as appropriate, and document the injury and investigation as a part of the resident's medical record.**

[Key note: If there is a reasonable suspicion of a crime having occurred related to the injury, covered individuals must report to the State Survey Agency and law enforcement under required timeframes.]

Section IV. Reportable Events Related to Potential Neglect

Alleged violations of neglect include cases where the facility demonstrates indifference or disregard for resident care, comfort or safety, resulting in physical harm, pain, mental anguish or emotional distress. There may be some situations in which the psychosocial outcome to the resident may be difficult to determine or incongruent with what would be expected. In these situations, it is appropriate to consider how a reasonable person in the resident's position would be impacted by the incident.

Examples of events to be reported include, but are not limited to, the following:

- Failure to meet payroll or pay supplier bills resulting in residents not receiving goods or services, such as:
    - Insufficient staff (including the night shift and weekends) resulting in the lack of provision for resident's care needs (e.g., residents who need continuous skilled nursing care or supervision, residents with cognitive deficits requiring continuous supervision); or
    - Lack of essential supplies or equipment such as incontinence supplies, wound care supplies, or oxygen equipment or adaptive equipment according to the needs of the resident(s); or
    - Lack of sufficient amounts of food to meet the residents' nutritional needs.
- Staff repeatedly ignoring residents' needs for assistance with activities of daily living, resulting in residents remaining in bed when they want to be up and repeatedly missing activities; or residents being left in fecal material or urine.
- Failure to oversee the management of pain for a resident resulting in a resident not receiving required medications or treatments, leading to prolonged excruciating pain.
- Failure to implement and monitor care planned interventions, resulting in repeated failures to provide necessary care and services to prevent the development a new avoidable pressure ulcer that develops into a Stage 3 or 4 pressure ulcer.

Section V. Reportable Allegations of Misappropriation of Resident Property and Exploitation

Examples of allegations of misappropriation of resident property and exploitation that must be reported include, but are not limited to:

- Theft of personal property, including but not limited to jewelry, computer, phone, and other valuable items such as eyeglasses and hearing aids;
- Unauthorized/coerced use by staff of resident's personal property;
- Theft of money from bank accounts;
- Unauthorized or coerced purchases on a resident's credit card;
- Unauthorized or coerced purchases from resident's funds;
- Staff who accept money from a resident for any reason including when staff have made the resident believe that staff was in a financial crisis or the resident believes that he/she is in a relationship with the staff person;
- A resident who provides a gift to staff in order to receive ongoing care, based on staff's persuasion; and
- Missing prescription medications or diversion of a resident's medication(s), including, but not limited to, controlled substances for staff use or personal gain.

Examples of allegations that would not be reported are:

- Theft of nominal items with little to no monetary or sentimental value;
- Lost items that are not listed under "must be reported."

Section VI. Reportable Allegations of Mistreatment

Allegations of mistreatment should be reported only if they meet the criteria for reporting alleged violations of abuse and/or exploitation, which are described under the Sections above.

Appendix C:
CMS SOM Quick Reference Abuse Reporting Guides
General Reporting Requirement Chart

Regulation	42 CFR 483.12(b)(5) and Section 1150B of the Act	42 CFR 483.12(c)
What is to be reported	Any reasonable suspicion of a crime against a resident or an individual receiving care from the facility	1) All alleged violations of abuse, neglect, exploitation or mistreatment, including injuries of unknown source and misappropriation of resident property 2) The results of all investigations of alleged violations
Who is required to report	Any covered individual, which means the owner, operator, employees, manager, agent or contractor of the facility	The facility
To whom	State Survey Agency (Department of Health Regional Office) *and* local law enforcement at a minimum.	The facility administrator and to other officials in accordance with State law, including to the SA and the adult protective services where state law provides for jurisdiction in long-term care facilities *NY and PA SNFs must report to the Department of Health Regional Office *PA must also report to the local Area Agency on Aging and Department of Aging in accordance with PA ACT 13 of 1997[1]
When	Serious bodily injury- Immediately but not later than 2 hours* after forming the suspicion No serious bodily injury- not later than 24 hours*	All alleged violations- 1) Immediately but not later than 2 hours*- if the alleged violation involves abuse or results in serious bodily injury 2) Not later than 24 hours*- if the alleged violation involves neglect, exploitation, mistreatment, or misappropriation of resident property; and does not result in serious bodily injury
		Results of all investigations of alleged violations- within 5 working days of the incident

*- Reporting requirements under this regulation are based on real (clock) time, not business hours.

[1]Act 13 of 1997 requires that PA employees and/or administrators who have reasonable cause to suspect that a recipient is a victim of any of the types of abuse described in the definitions of Act 13 shall immediately make an oral report to the AAA. Within 48 hours of making all oral reports, the employee or administrator shall make a written report (may use PB-22) to the AAA. Any alleged abuse involving sexual abuse, serious bodily injury, serious physical injury and suspicious death is mandated under Act 13 to be reported to the Pennsylvania Department of Aging (PDA), in addition to the AAA report.